Behavioral Health Agency Inspection Report

Department of Health
P.O. Box 47874, Olympia, WA 98504-7874
TEL: 360-236-4732

January 31, 2025

Newport Academy - Seattle, 12029 113th Ave NE, Kirkland, WA. 98034-6901	David Laxton
Agency Name and Address	Administrator

ONGOING - ROUTINE	12/03/24	ECR03
Inspection Type	Inspection Onsite Dates	Inspector
X2024-958	BHA.FS.61015899	
Inspection Number	License Number	

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the on-site inspection.

Deficiency Number and Rule Reference	Observation Findings	Plan of Correction
0705 Personnel-Agency record requirements WAC 246-341-0510(1)(c)(v) A behavioral health agency must maintain a personnel record for each person employed by the agency. (1) The personnel record must contain all of the following: (c) A record of an orientation to the agency within 90 days of hire that includes all of the following: (v) Violence prevention training on the safety and violence prevention topics described in RCW 49.19.030.	Based on interview and personnel record review, the agency failed to ensure all staff had initial violence prevention training in the personnel record. Failure to ensure timely training may result in client harm. Findings included: 1. Review of the personnel record for Staff A, Mental Health Clinical Supervisor, and Staff B, Administrative Assistant, showed that there was no initial violence prevention training in their personnel file.	0705 Personnel-Agency record requirements WAC 246-341-0510(1)(c)(v) Deficiency will be corrected as followed: Human Resources will perform monthly audits on staff initial violence prevention trainings in the personnel record and provide findings to Residential Leadership Team (Program Manager, Executive Director, Clinical Director). All initial violence prevention trainings are completed within the first 30 days for all new hires. Initial violence

	2. During an interview with Staff E, Human Resources (HR) Generalist, on 12/03/24 at 12:58 PM, they stated, “I don’t see one in their first 90 days.”	prevention trainings are conducted twice a month. Please see attached Policy.
0715 Personnel-Agency record requirements WAC 246-341-0510(1)(d)(i) A behavioral health agency must maintain a personnel record for each person employed by the agency. (1) The personnel record must contain all of the following: (d) A record of annual training that includes: (i) Cultural competency.	Based on interview and personnel record review, the agency failed to ensure all staff had ongoing training in the personnel record. Failure to ensure ongoing training may result in client harm. Findings included: 1. Review of the personnel record for Staff B, Administrative Assistant, showed that there were no annual cultural competency trainings in their personnel file. 2. During an interview with Staff E, HR Generalist, on 12/03/24 at 11:45 AM, they stated, “I don’t see any cultural competency trainings in the file.”	0620 Management of human resources WAC 246-337-050(5)(a) Deficiency has been corrected as followed: Cultural Competency Trainings have been identified in staff files and titled “Implicit Bias.” Transcripts for Staff B have been attached for confirmation of cultural competency training in personnel file. Human Resources has been educated on training module titles and their contents. In July 2024, the organization switched to Relias Training Program and implemented an Education Fair in which all required annual trainings are automatically assigned to all staff.
0925 Individual service record content WAC 246-341-0640(1)(a) A behavioral health agency is responsible for the components and documentation in an individual's individual service record content unless specified otherwise in certification or individual service requirements. (1) The individual service record must include: (a) Documentation the individual received a copy of counselor	Based on clinical record review and interview, the agency failed to provide documentation that individuals received a copy of counselor disclosure requirements as required for the counselor credential for 4 of 4 clinical records reviewed (Client #1-#4). Failure to ensure all clients receive the counselor disclosure requirements and show documentation in the clinical record may result in clients not choosing	0925 Individual service record content WAC 246-341-0640(1)(a) Deficiency will be corrected as followed: Discharge Letters are signed and provided to each client upon discharge. We have submitted a request to our EHR to add in a field for treatment team

disclosure requirements as required for the counselor's credential.	counselors who best suit their needs and can result in poor client outcomes. Findings included: 1. Review of the clinical record for Client #1, Client #2, Client #3, and Client #4 showed they did not contain a counselor disclosure as required for the counselor's credential. 2. During an interview with Staff E, Administrator, on 12/03/24 at 2:28 PM, they stated, "We have it on the wall, just not a detailed copy for the clients."	names and credentials on this letter.

Introduction
We require that you submit a plan of correction for each deficiency listed on the inspection report form. Your plan of correction must be submitted to the DOH within ten business days of receipt of the list of deficiencies.

Descriptive Content
Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency;
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited;
- The monitoring procedure to ensure that the plan of correction is effective and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements;
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

Completion Dates
The POC must include a completion date that is realistic and coinciding with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable time-frame.

Continued Monitoring
Each plan of correction must indicate the appropriate person, either by position or title, who will be responsible for monitoring the correction of the deficiency to prevent recurrence.

Checklist:

- Before submitting your plan of correction, please use the checklist below to prevent delays.
- Have you provided a plan of correction for each deficiency listed?
- Does each plan of correction show a completion date of when the deficiency will be corrected?
- Is each plan descriptive as to how the correction will be accomplished?
- Have you indicated what staff position will monitor the correction of each deficiency?
- If you included any attachments, have they been identified with the corresponding deficiency number or identified with the page number to which they are associated?

Your plan of correction will be returned to you for proper completion if not filled out according to these guidelines.

Note: Failure to submit an acceptable plan of correction may result in enforcement action.

Approval of POC

Your submitted POC will be reviewed for adequacy by DOH. If your POC does not adequately address the deficiencies in your inspection report you will be sent a letter detailing why your POC was not accepted.

Questions?

Please review the cited regulation first. If you need clarification, or have questions about deficiencies you must contact the inspector who conducted the onsite inspection, or you may contact the supervisor.

STATE OF WASHINGTON

DEPARTMENT OF HEALTH

2/18/2025

Newport Academy - Seattle
12029 113th Ave NE
Kirkland, WA 98034-6901

Re: Exam Number: X2024-958
License Number: BHA.FS.61015899
Approved Plan of Correction
Date(s) of Inspection: 12/03/24

Dear Administrator:

This letter is to inform you that after careful review of the Plan of Correction (POC) you submitted for the exam recently conducted at your facility, the Department has determined that the POC is approved.

Based on the scope and severity of the deficiencies listed in your statement of deficiency report, the Department **will not** conduct an announced follow-up compliance visit to verify that all deficiencies have been corrected.

Reviewer: ECR03
Department of Health
HSQA/Office of Health Systems Oversight
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